4.10 Infective Endocarditis*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I have had fever and sweats for 2 weeks." **Objective** • Temperature 38.6 °C • New murmur • Positive blood cultures	**ACTUAL** Hyperthermia related to infection of the cardiac valves as evidenced by fever and positive blood cultures.	• Afebrile within 72 hours of starting antibiotics.	• Take blood cultures before antibiotics, as ordered. • Give IV antibiotics on time, for the whole course. • Measure temperature every 4 hours. • Maintain long-term IV access.	• Considered met once he is afebrile.
Subjective • "I get tired quickly." **Objective** • Fatigue • Anemia	**ACTUAL** Decreased activity tolerance related to infection and anemia as evidenced by fatigue.	• Manages ADL with rests and stable vital signs within 1 week.	• Plan rest between activities. • Increase activity as the fever settles. • Check HR and BP during activity. • Follow hemoglobin.	• Goal reached when ADL are tolerated.
Subjective *No specific complaint* **Objective** • Valve vegetations • Large vegetation on echocardiography	**RISK** Risk for ineffective cerebral tissue perfusion as evidenced by risk factors: embolization of vegetations.	• No stroke or other embolic event.	• Assess neurological status and the limbs; test urine for blood. • Report sudden neurological signs, limb pain or chest pain.	• Met if no emboli occur.

* **References for this care plan:** [16], [17], [32], [35]. Full entries are listed in the References section of the CPMN book.